

1) Device & Lead Details

Component	Model	Serial	Placement	Initial Values
Pulse Generator	PulseSync AtriaVita DDD-200	PG-AVD200-11837	Right pectoral pocket	Mode DDD; Base rate 60 bpm
Atrial Lead	PS-A52	AL-523741	RA appendage	P-wave 3.4 mV; Threshold 0.6 V @ 0.4 ms; Impedance 520 Ω
Ventricular Lead	PS-V60	VL-603219	RV septal	R-wave 9.8 mV; Threshold 0.7 V @ 0.4 ms; Impedance 640 Ω

2) Intra/Post-Op Parameters

Parameter	Value	Reference/Target	Status
Pocket Hemostasis	Achieved	Dry	Normal
Lead Stability (fluoro)	Stable	No dislodgement	Normal
Final Pacing Thresholds	A: 0.5 V @ 0.4 ms V: 0.7 V @ 0.4 ms	≤ 1.0 V @ 0.4 ms	Good
Impedance (ohms)	A: 510 Ω V: 630 Ω	300–1200	Normal
Sensing	P-wave 3.2 mV R-wave 10.1 mV	> 2 mV (A), > 5 mV (V)	Normal
Immediate CXR	No pneumothorax; leads appropriate trajectory	Clear	Normal

3) Programming Summary

Mode: DDD; Lower rate: 60 bpm; Upper tracking: 130 bpm; AV delays: 150/180 ms; Rate response: Enabled (moderate).

4) Device-detected Arrhythmias (Day 0–1)

No sustained atrial or ventricular arrhythmias detected. PVC burden low without runs. No mode-switch events. Symptoms of presyncope have resolved post-implant.

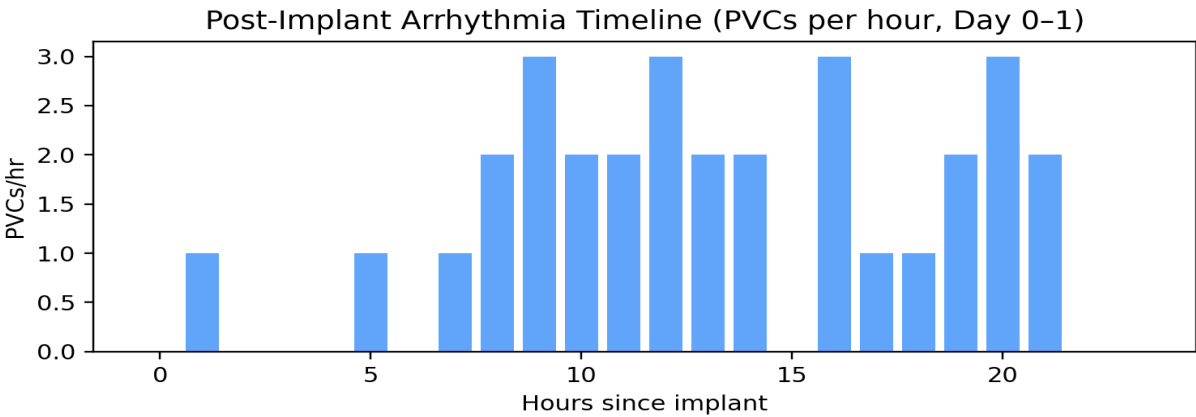


Figure D: Arrhythmia timeline shows low PVC count per hour and absence of significant tachyarrhythmias (synthetic).

5) Telemetry Baseline (Day 0-1)

Metric	Value	Status
Pacing % (A/V)	12% / 58%	Expected
Battery Voltage / Est. Longevity	3.02 V / ~9.8 years	Normal
Lead Impedance Trend	Stable ($\pm 3\%$)	Normal
Events	None sustained; occasional PVCs	Benign

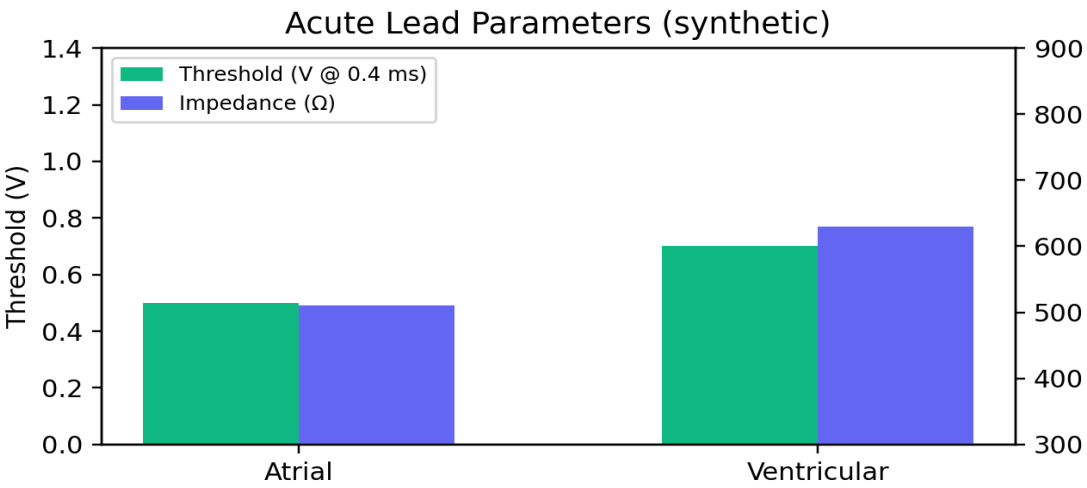


Figure C: Acute lead thresholds and impedance within targets, supporting adequate capture and sensing margins (synthetic).

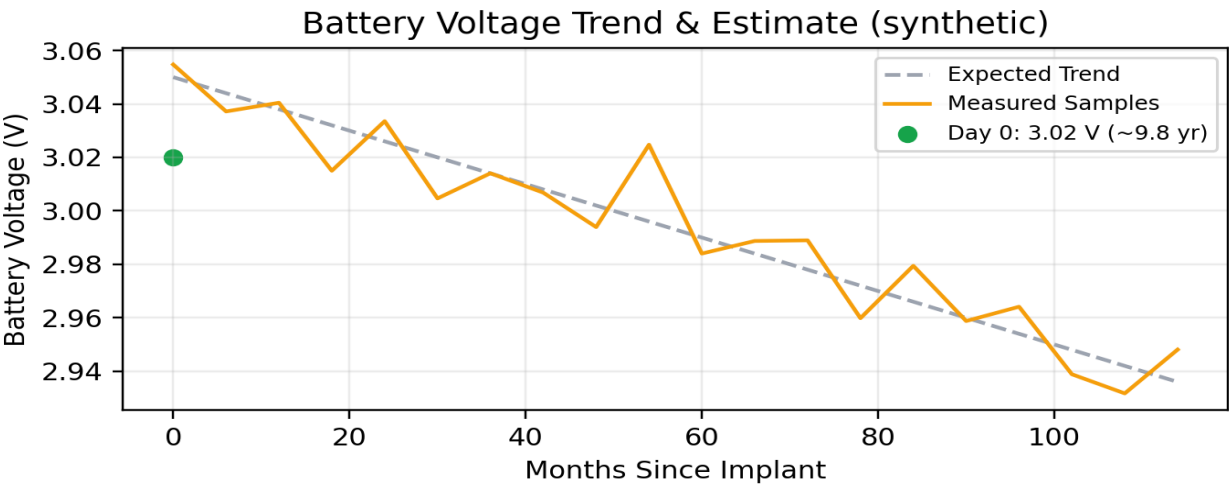


Figure E: Battery voltage trajectory with day-0 baseline aligns with expected decay model (synthetic).

6) Recovery Status (Issues Resolved)

Indication-related issues have improved: symptomatic bradycardia and high-grade AV block resolved with appropriate pacing support. No presyncope reported. Incision site clean and intact.
Follow-up plan: remote transmission in 48-72 h; wound check at 7-10 days; comprehensive device check at 6 weeks.
Disclaimer: Charts and values are synthetic for demonstration and not for clinical use.

Post-Implant Medications for the Patient to Follow

1. Antibiotics

- Cephalexin 500 mg orally every 6 hours for 5 days
- If penicillin-allergic: Clindamycin 300 mg every 8 hours for 5 days

2. Pain Management

- Acetaminophen 500–650 mg every 6 hours as needed
- Avoid NSAIDs for 7–10 days unless cleared by the cardiology team

3. Cardiac Medications

- Continue beta-blockers, antihypertensives, or antiarrhythmics as instructed

4. Anticoagulation / Antiplatelet Guidance

- Resume anticoagulants 12–24 hours post-procedure depending on bleeding risk
- Resume aspirin or clopidogrel the next day unless advised otherwise

5. Wound Care Adjuncts

- Topical mupirocin only if instructed; no creams on the incision site